

System	AegisCare HMS / LabTrack v5.5
Repository	fmurage6331-dev/confit-core
Live URL	https://aegiscarehms.lovable.app
Database	tgynjasgnerucrwlwedu
Project name	Self-attestation gap analysis for DHA HIE certification (Form HMIS 4 pathway)
Document	Kenya Digital Health Act, 2023; Digital Health (Data Exchange Component) Regulations, 2025; FHIR R4; ICD-11; Social Health Insurance Act, 2023 (esp. s.48); Kenya Data Protection Act, 2019; SHA benefit package framework standards (Legal Notice 146 & 147 of 2024)
DHA HIE API base	https://ilm-dev.dha.go.ke/uat-middleware/api/v1 (per hie-docs.dha.go.ke)
Prepared	2026-08-12
Status	DRAFT for developer review before submission

Attestation scope note. This analysis was cross-checked against the DHA HIE API catalogue published at `https://hie-docs.dha.go.ke/catalog` (Authentication, HIE Registries, eClaims & Preauth, Consent Services, SHR Service APIs, Terminology Service). The attachments referenced in the briefing did not arrive with the task brief; the developer should re-verify section-by-section against the official certification checklist (Form HMIS 4, `https://certification.dha.go.ke/process`) and the printed DHA documents before submission.

Summary of Findings

#	Section	Status
1	FHIR R4 Compliance	PARTIAL — resources built; DHA profile conformance & Bundle submission not yet demonstrated
2	Patient Identity (Client Registry)	PARTIAL — schema + UI ready; CR lookup blocked on credentials
3	SHA Claims Workflow	PARTIAL — full local pipeline built; live submission blocked on credentials
4	Security & Access Control	COMPLIANT (with two P2 items: MFA, server-side password policy)
5	Audit & Logging	COMPLIANT (20-year retention procedure to be finalised)
6	Data Protection	PARTIAL — controls in place; ODPC registration & DPO pending
7	Preauthorisation Workflow	PARTIAL — local enforcement built; DHA preauth API blocked on credentials
8	Pharmacy & NLMIS	PARTIAL — coding complete (297/331); NLMIS reporting submission is a build gap
9	Laboratory	COMPLIANT for internal workflow; FHIR Observation mapping is a build gap
10	Outstanding Items (P0)	PENDING — blocked on credentials

Legend: ☒ COMPLIANT = implemented and demonstrable · ☐ PARTIAL = implemented in part; a defined gap remains · ☐ PENDING = blocked on external credentials · ☐ GAP: = build gap to close.

Section 1 — FHIR R4 Compliance

DHA requirement

The DHA Shared Health Record (SHR) accepts FHIR R4 Bundle resources (POST /shr/bundles) containing Encounter, EpisodeOfCare, Observation, Condition, Procedure, Medication, MedicationRequest, MedicationDispense, MedicationAdministration, Immunization, ServiceRequest and AllergyIntolerance. Every Encounter must reference the visit's EpisodeOfCare, and all clinical resources must reference that Encounter. Bundles must carry an id (echoed as mediator_id) and be of type collection (source: <https://hie-docs.dha.go.ke/sharedhealthrecord/shr-records>).

What AegisCare implements

- generate_fhir_encounter(p_encounter_id UUID) — PL/pgSQL RPC (supabase/migrations/20260805000000_sprint52_sha_fhir_foundation.sql, enhanced by 20260806000004_sprint8e_fhir_second_schedule.sql). Emits a FHIR R4 Encounter with identifier (facility encounter ID + SHA notification number), class (AMB/IMP/EMER via v3-ActCode), serviceType (SHA fund PHF/SHIF/ECCIF), subject, participant (practitioner + council registration identifier), period, diagnosis (ICD-11 with rank), hospitalization (referral block), serviceProvider (KMHFL identifier) and DHA extensions (sha-notification-number, sha-fund-type, clinical-findings, blood-group, allergies).
- Edge functions (all verify_jwt = true in supabase/config.toml):
 - supabase/functions/fhir-patient/index.ts — FHIR R4 Patient
 - supabase/functions/fhir-encounter/index.ts — FHIR R4 Encounter
 - supabase/functions/fhir-condition/index.ts — FHIR R4 Condition[] from encounter_diagnoses (verificationStatus mapping, ICD-11 coding with <http://id.who.int/icd/release/11/mms>)
- trg_medication_dispense_fhir() trigger function (20260811000001_sprint12d_medication_dispense_fhir.sql) — writes a FHIR R4 MedicationDispense payload to dha_outbound_queue on dispense.
- ICD-11 structured diagnosis layer — encounter_diagnoses + trg_sync_encounter_diagnoses (20260730120000_dha_icd11_encounter_diagnoses_sync.sql).

Compliance status: PARTIAL

What is missing / required before submission:

1. DHA profile conformance testing — output of generate_fhir_encounter() and the three FHIR edge functions has not yet been validated against the DHA FHIR Implementation Guide / StructureDefinitions. Validate at <https://validator.fhir.org> and against the DHA profiles, and attach the validation report to the application. (Not blocked by credentials — build/QA task.)
2. Bundle assembly (collection type) — no function currently assembles the Patient + Encounter + Condition (+ MedicationDispense) Bundle required by POST /shr/bundles. FhirSyncHandler in claims-dispatcher queues a payload but does not yet build the Bundle.  GAP: build the Bundle builder (recommended: generate_fhir_bundle(encounter_id) RPC or a fhir-bundle edge function) before live submission.
3. EpisodeOfCare — the DHA SHR requires the Encounter to reference an EpisodeOfCare; AegisCare does not yet generate one.  GAP: add EpisodeOfCare generation (SHA visit-based) to the Bundle builder.
4. Live SHR submission —  PENDING: blocked on DHA OAuth2 credentials (POST /tenants/token), AfyaLink sandbox access, and the per-visit consent-token flow (POST /shr/consents, POST /shr/consents/{consent_id}/verify).

- 5. Observation resources — laboratory results are not yet mapped to FHIR Observation (see Section 9).

Section 2 — Patient Identity (Client Registry)

DHA requirement

HIE Client Registry API GET /patients?identification_number=...&identification_type=... (identification types: National ID, ClientRegistry ID, Birth Notification, Birth Certificate, Alien ID, Refugee ID, Mandate Number). The registry returns the canonical client record including the Client Registry (CR) ID, which is then used for eligibility (/patients/benefits, /patients/eligibility), SHR consent (cr_id) and claims. DHA certification expects facilities to resolve and persist the CR ID (UPI-equivalent) for SHA patients (source: <https://hie-docs.dha.go.ke/registries/client-registry>).

What AegisCare implements

- patients.national_id, patients.national_id_type (CHECK: national_id | passport | birth_certificate | alien_id), patients.cr_number (comment: *"DHA Client Registry number — retrieved via CR lookup API"*), patients.identity_verified, identity_verified_at, identity_verified_by (20260812000004_drift_patients_sha_columns.sql, 20260812000010_sha2_claims_tables.sql).
- verify_patient_identity() PL/pgSQL stub RPC (called from src/routes/register-patient.tsx verifyIdentity(), line ~234), wired to the "Verify identity & SHA status" button on the registration form.
- Consent-gated invocation: ConsentDialog (src/components/consent-dialog.tsx) with OTP flow before any registry lookup; consent_otps (SHA-256 hashed, 10-minute expiry, consent_type IN ('patient_consent','sha_claim','preauth','shr_access')).
- FHIR Patient identifier block already emits <https://iprs.go.ke/ns/national-id> (NI/PPN/BCT) in generate_fhir_encounter().

Compliance status: PARTIAL

-  PENDING: live Client Registry lookup requires DHA developer-account credentials (client_id/client_secret for POST /tenants/token) and sandbox access via <https://sandbox.dha.go.ke> per docs/integration-activation-manual.md (Integration 1 — IPRS Patient Identity).
-  GAP: cr_number is never auto-populated today because the lookup is a stub; once credentials arrive, the verify-identity edge function (to be created per the activation manual) must persist the CR ID, and the FHIR Patient.identifier block should include <https://dha.go.ke/ns/cr-id>.
- The current on-screen OTP (Track A) satisfies the consent-evidence requirement in the interim; SMS delivery (Track B via Africa's Talking) is  PENDING credentials.

Section 3 — SHA Claims Workflow

DHA requirement

SHA electronic claims are submitted by facilities under the SHA claims API (eClaims & Preauth catalogue), keyed to the SHA notification number issued at visit initiation, the member number, the Client Registry ID, the facility/provider number, and structured ICD-11 diagnoses; preauthorisation is mandatory for listed interventions (e.g., dialysis, oncology, ICU, elective surgery per the SHA benefit package framework and Legal Notices 146/147).

What AegisCare implements

- Claims schema (new this session, live in the database):
 - sha_claims — encounter_id, patient_id, claim_number, dha_claim_id, claim_type (sha_shif|sha_phf|sha_eccif|private|cash), fund_type (PHF|SHIF|ECCIF), status (draft → pending_otp → pending_preauth → ready → submitted → acknowledged → approved | rejected → appealed → paid), preauth_number/preauth_status/preauth_requested_at/preauth_approved_at, otp_verified, consent_token, total_amount, fhir_bundle, timestamps (20260812000010_sha2_claims_tables.sql).
 - sha_claim_items — line items sourced from invoice_line_items (item_type, description, quantity, unit_price, amount, intervention_code, is_included).
 - sha_claim_packages — benefit-package assignment per claim (unique per claim_id + package_code, is_primary).
 - sha_benefit_packages — 22 seeded packages across PHF (3), SHIF (15), ECCIF (4) with requires_preauth, can_combine_with, daily_limit, annual_limit, per_visit_limit (20260812000009_sha1_benefit_packages.sql).
- Automatic claim generation: auto_generate_sha_claim() trigger function + trg_auto_generate_sha_claim (AFTER UPDATE OF status ON encounters) — on status = 'signed' with payment_mode = 'insurance' it creates a draft sha_claims row and seeds line items from the encounter invoice (20260812000011_sha4_auto_claim_trigger.sql).
- Encounter-level claim tracking: encounters.sha_notification_number (placeholder TEMP-SHA-{ts}-{id} until live), insurer_type (sha_shif|private|corporate), claim_number, claim_status, claim_submitted_at, claim_resolved_at, preauth_number, sha_fund_type (auto-detected by set_sha_fund_type() / trg_set_sha_fund_type — emergency→ECCIF, inpatient→SHIF, else PHF).
- Insurance Desk UI: src/routes/rooms.\$id.tsx (room kind insurance) — submitClaim() (~line 2780) saves packages/line items, marks the claim ready, inserts into dha_outbound_queue (queue_type = 'sha_claim' or 'private_claim') with the FHIR encounter payload, and sets claim_status = 'submitted'.
- Dispatcher: supabase/functions/claims-dispatcher/index.ts — ShaClaimsHandler, PrivateClaimsHandler, CashReceiptHandler, FhirSyncHandler (deployed, stub mode — queues locally only).
- Monitoring: src/routes/admin.queue.tsx — dual-rail claims queue monitor with claims aging and FHIR preview.
- Tariffs: sha_tariffs seeded with 18 tariff lines from Legal Notices 146/147 (20260805000000_sprint52_sha_fhir_foundation.sql).

Compliance status: PARTIAL

-  COMPLIANT (local): end-to-end internal claim pipeline — sign → auto-generate draft → package/line-item selection → OTP consent evidence → queue → status tracking — is implemented and demonstrable in the live deployment.
-  PENDING (credentials): actual submission to SHA — OAuth2 credentials, SHA provider number, sandbox URL (<https://api.sandbox.sha.go.ke/v1> per the activation manual), real notification numbers, and dha_claim_id assignment.
-  GAP: ShaClaimsHandler must be upgraded from stub to live (POST {SHA_API_URL}/claims/submit) once credentials arrive; the activation steps are already documented in docs/integration-activation-manual.md (Integration 4).
-  GAP: the fhir_bundle column on sha_claims is populated only when the insurance desk generates a preview; a deterministic claim-bundle builder is recommended (see DOC-5 Section 5 — "SHA-10 pending").

Section 4 — Security & Access Control

DHA requirement

DHA certification requires role-based access control, least-privilege operation, secure authentication (including automatic session termination), password policy, and protection of clinical data from unauthorised access (DHA security scoring-tool line items; Digital Health (Data Exchange Component) Regulations 2025).

What AegisCare implements

- 19 roles (src/lib/roles.ts): admin, system_admin, staff, receptionist, accountant, insurance_agent, records_officer, triage_nurse, nurse, doctor, clinical_officer, dental_officer, nutritionist, physiotherapist, hts_counsellor, lab_tech, radiologist, pharmacist, mortician — matching the app_role enum (20260716063555... + 20260730140000_dha_task_d_security_roles.sql).
- is_approved(uuid) — SECURITY DEFINER, SET search_path TO 'public'; single gate covering all 19 roles (fixes the 68-policy role gap); used by ~134 RLS policy definitions across the schema.
- has_role(), user_has_permission(), role_permissions table and src/lib/require-access.tsx (Guard, PermGuard), hasPerm() in src/lib/auth-context.tsx.
- Admin-only user management via server functions (src/lib/admin-users.functions.ts, requireSupabaseAuth middleware, assertAdmin) — service-role key never reaches the browser.
- Session timeout: 30 minutes inactivity auto-logout (src/components/app-shell.tsx, TIMEOUT_MS = 30 * 60 * 1000).
- Password policy: min 8 chars, uppercase, number, special char, max 72 (src/routes/change-password.tsx, Zod).
- Security hardening migration 20260811000007_security_hardening.sql: revoked anon EXECUTE on 8 SECURITY DEFINER functions; pinned search_path on 12 functions; restricted audit_log INSERT to authenticated; restricted profiles and moh_705_disease_mappings SELECT to authenticated; rebuilt stock_store_usage_view without the auth.users join.
- RLS enabled on all clinical tables, including lab_orders, lab_results, encounter_room_visits, icd11_codes, moh_indicator_definitions, room_indicator_map (20260806000005_rls_missing_tables.sql).

Compliance status: ☒ COMPLIANT (with two deferred items)

- ☒ COMPLIANT: RBAC, least privilege, RLS, session timeout, password policy, admin-only provisioning.
- ☐ PENDING / GAP (low): MFA enforcement — Supabase Auth MFA requires a paid plan / own Supabase account (documented in docs/dha-compliance-task-d-phase2-security.md); server-side password policy — requires Supabase dashboard access unavailable on the current Lovable free plan. Both are scheduled for transfer to the facility's own Supabase project.

Section 5 — Audit & Logging

DHA requirement

Immutable, complete audit of clinical data changes, security/access changes and pharmacy/stock movements; a separate SHR transmission metadata log is a statutory obligation under the Digital Health (Data Exchange Component) Regulations 2025; long-term (20-year) retention is expected for audit records.

What AegisCare implements

- `audit_log` (append-only RLS: `audit_log_insert_authenticated`; UPDATE/DELETE denied by `audit_log_deny_update` / `audit_log_deny_delete`).
- `audit_trigger_fn()` — generic trigger function (SECURITY DEFINER, `search_path = public`); 20 tables / 60 triggers audited (Task E, 20260730150000_dha_task_e_audit_completeness.sql): admissions, encounters, invoice_line_items, invoice_payments, invoices, lab_tests, patients, prescriptions, lab_orders, lab_results, radiology_orders, radiology_results, clinical_notes, encounter_diagnoses, user_roles, role_permissions, user_room_access, app_settings, stock_movements, stock_items.
- Break-glass: `log_break_glass_access()` writes BREAK_GLASS audit events and queues a client notification (20260806000001_sprint8a_break_glass.sql, 20260806000002_sprint8c_shr_access_notification.sql).
- Archiving: `audit_log_archive`, `audit_archive_runs`, `archive_old_audit_logs()` (2-year hot-retention cutoff), cron `archive-audit-logs-nightly` (0 23 * * * = 02:00 EAT) (20260806101050_e8638a1f....sql).
- SHR transmission log: `shr_transmission_log` (append-only, RLS read-only for users) written by `trg_shr_transmission_log` / `trg_shr_transmission_log_insert` on `dha_outbound_queue` for `fhir_sync` and `shr_access_notification` types (20260806000003_sprint8d_shr_transmission_log.sql).
- UI: `src/routes/admin.audit-log.tsx`.

Compliance status: ☒ **COMPLIANT (one administrative item)**

- ☒ **COMPLIANT**: 20-table / 60-trigger coverage; append-only; break-glass; SHR transmission metadata log separate from the general audit log.
- ☐ **GAP (administrative)**: a documented 20-year audit retention schedule and legal-hold procedure must be written and attached (the archiving pipeline currently moves records older than 2 years into `audit_log_archive`, which is retained indefinitely — the retention statement itself needs to be formalised; see DOC-4 and DOC-3 Section 6).

Section 6 — Data Protection

DHA requirement

Compliance with the Kenya Data Protection Act, 2019 (and the Digital Health Act, 2023): lawful basis, consent management, purpose limitation, data minimisation, security safeguards, data-subject rights, breach notification, and registration with the Office of the Data Protection Commissioner (ODPC).

What AegisCare implements

- Consent management: `patient_consents` (live table; columns include `consent_type`, `consented`, `hie_data_sharing_consented`, `consented_at`, `consented_by`) written by `ConsentDialog` after OTP verification; `consent_otps` stores only SHA-256 hashes of OTPs, 10-minute expiry, and the SHA Act 2023 s.48 disclaimer text is embedded in the UI and the SMS template (`src/components/consent-dialog.tsx`).
- SHR access notification: `trg_shr_access_notification()` queues an alert to the client whenever a new encounter accesses their record, and on break-glass access (`20260806000002_sprint8c_shr_access_notification.sql`).
- Encryption: Supabase platform defaults — AES-256 at rest, TLS 1.2+ in transit (documented in `docs/dha-compliance-task-d-phase2-security.md`).
- Data minimisation: RLS + 19-role RBAC; untyped db client used only for columns absent from generated types (`src/lib/supabase-untyped.ts` — intended architecture).
- This documentation set: DOC-2 (DPIA), DOC-3 (Privacy Policy), DOC-6 (Security Policy) are the regulatory artefacts.

Compliance status: PARTIAL

-  PENDING: ODPC registration of `FACILITY_NAME` as data controller (registration number not yet obtained — placeholder `ODPC_REG_NO` in DOC-3).
-  PENDING: named Data Protection Officer (DPO) — placeholder in DOC-2/DOC-3.
-  GAP (administrative): staff privacy/security training records and a data breach register must be maintained; templates are referenced in DOC-6 Section 9.

Section 7 — Preauthorisation Workflow

DHA requirement

The SHA benefit framework requires preauthorisation for specified interventions (e.g., haemodialysis — KES 10,650/session, oncology, ICU/HDU, elective surgery, advanced radiology). The HIE eClaims & Preauth catalogue provides the preauth APIs; a claim for a preauth-required intervention must carry a valid preauth reference.

What AegisCare implements

- `sha_benefit_packages.requires_preauth` — true for SHA-16 (Renal Care — Dialysis), SHA-17 (Radiology — Advanced), SHA-19 (Surgical Services), SHA-20 (Oncology), SHA-21 (ICU / Critical Care); `can_combine_with` matrix seeded.
- Insurance Desk enforcement (`src/routes/rooms.$id.tsx`): preauth procedure keywords scanned against the encounter; `preauthMissing` blocks claim submission with an explicit warning; `preauth_number` is captured and persisted on the encounter and on `sha_claims.preauth_number` (also `preauth_status`, `preauth_requested_at`, `preauth_approved_at`).
- Consent type preauth supported in `consent_otps` CHECK constraint.
- `sha_claims.status` includes `pending_preauth` in the lifecycle.

Compliance status: **PARTIAL**

-  **COMPLIANT** (local): preauth capture, persistence and submission-blocking are implemented.
-  **PENDING**: DHA/SHA preauth API (request + status polling) blocked on credentials — upgrade path documented in docs/integration-activation-manual.md.
-  **GAP**: no local expiry check on preauth_number validity window (relies on sha_claims.preauth_status); recommend a validation rule when the live API is connected.

Section 8 — Pharmacy & NLMIS

DHA requirement

Medication coding aligned to the national logistics system (NLMIS / KEMSA commodity codes) for pharmaceutical supply-chain visibility, plus FHIR MedicationDispense visibility into the SHR.

What AegisCare implements

- stock_items.nlmis_code (20260810000001_sprint11c_nlmis_code.sql) — live coverage 297 of 331 stock items (89.7%); the remaining 34 items are legitimately untagged (ARVs, EPI vaccines, equipment — not NLMIS commodities).
- FHIR MedicationDispense: trg_medication_dispense_fhir() on prescriptions (status → dispensed) writes a FHIR R4 MedicationDispense (medicationCodeableConcept with <https://kemsa.go.ke/ns/nlmis-code>, subject, context, performer, quantity UCUM, dosageInstruction, DHA extensions) to dha_outbound_queue (20260811000001_sprint12d_medication_dispense_fhir.sql).
- Stock integrity: dispense_prescription_stock() negative-stock guard (Sprint 13B, 20260811000003_sprint13b_dispense_stock_guard.sql), apply_stock_movement(), trg_smov_apply, delivery_to_stock(), trg_deliv_stock, stock movements audited.
- MAR: medication_administrations (nurse-recorded administrations) with RLS (20260811000005_sprint14c_medication_administrations.sql).
- MOH 707 pharmacy report route (src/routes/moh.707.tsx).

Compliance status: **PARTIAL**

-  **COMPLIANT**: NLMIS coding coverage and MedicationDispense FHIR generation.
-  **GAP** (build): no NLMIS reporting/submission integration (KHIS/KEMSA) — the system currently reports pharmacy aggregates through MOH 707 only; the NLMIS reporting interface must be built or the exemption justified to DHA.
-  **PENDING**: SHR transmission of MedicationDispense blocked on AfyaLink credentials.

Section 9 — Laboratory

DHA requirement

Structured, verifiable laboratory workflows feeding the SHR (FHIR Observation / DiagnosticReport) and supporting MOH 706 reporting; specimen traceability and critical-result handling are expected good practice for certification.

What AegisCare implements

- Order-driven worklist: lab_orders (order_number ORD-#####, priority routine|urgent|stat, status ordered → in_progress → completed | declined) auto-created by sync_lab_orders_from_tests() / trg_sync_lab_orders_from_tests from encounters.tests (20260721120000_laboratory_orders.sql).
- Results: lab_results (structured result JSONB, performed_by, reported_at) — entered in src/routes/laboratory.\$id.tsx.
- Specimen & critical handling (Sprint 13E, 20260811000004_sprint13e_lab_specimen_critical.sql): lab_orders.specimen_type, collected_at, is_critical; lab_results.is_critical, verified_by, verified_at — dual-verification UI in laboratory.\$id.tsx ("Verify result" action).
- Routing: send_lab_result_to_room() / send_lab_results_to_requesting_room() return results to the requesting room; radiology equivalent send_radiology_results_to_requesting_room() (20260728120000_radiology_room_routing.sql).
- RLS: full CRUD policies for authenticated approved staff (20260806000005_rls_missing_tables.sql); lab tables audited (Task E).
- MOH 706 report route (src/routes/moh.706.tsx).

Compliance status: ☒ COMPLIANT (internal) with one build gap

- ☒ COMPLIANT: order/result lifecycle, specimen capture, critical alerts, dual verification, routing, audit.
- ☐ GAP (build): no FHIR Observation/DiagnosticReport generation for lab results yet — required for full SHR write coverage; recommend a trg_lab_result_fhir mirror of trg_medication_dispense_fhir (see DOC-5 Section 9).

Section 10 — Outstanding Items (P0, blocked on credentials)

All P0 items below are architected, stubbed and documented in docs/integration-activation-manual.md; none can be activated until the listed credentials/access are obtained. None of these are code-quality gaps.

#	Item	Status	Blocking credential
1	DHA OAuth2 client (POST /tenants/token)		DHA developer account (https://developer.dha.go.ke)
2	Client Registry / IPRS verification (verify_patient_identity → live)	PENDING	IPRS client ID/secret, sandbox access
3	SHA member registry verification (patients.sha_membership_status → live)	PENDING	SHA provider portal credentials
4	SHA notification numbers (replace TEMP-SHA- placeholders)	PENDING	SHA eligibility API credentials
5	SHA claims submission (ShaClaimsHandler → live)	PENDING	SHA claims API credentials + provider number
		PENDING	

#	Item	Status	Blocking credential
6	AfyaLink HIE sandbox (FHIR Bundle → POST /shr/bundles)	PENDING	DHA Provisional Interoperability Accreditation + AfyaLink secrets
7	Kenya HWR practitioner verification (verify_practitioner → live)		HWR API key
8	WHO ICD-11 API (icd11-search live mode)	PENDING	ICD_CLIENT_ID / ICD_CLIENT_SECRET (function is correct; local icd11_codes table is the working fallback)
9	SMS OTP delivery (Track B)	PENDING	Africa's Talking API key + CA sender-ID whitelist
10	M-Pesa (Daraja)	PENDING	Safaricom consumer key/secret (planned; not built)

Recommended order of unblocking: 1 → 2/3 → 4 → 5 → 6 → 7 → 8 → 9, following the activation manual's golden rule (test in sandbox before production).

Attestation

I/we attest that the information in this gap analysis reflects the state of the AegisCare HMS / LabTrack v5.5 codebase at commit be795c10 (branch main) and the live Supabase project tgynjasgnerucrlwedui, and that all PARTIAL/PENDING items are disclosed to the certifying authority.

Prepared by

(Developer / Technical Lead)

Reviewed by

(Facility In-Charge)

Date

End of DOC-7.